SUSPECTED ADVERSE DRUG REACTION REPORTING FORM

Pharmacovigilance Programme of India (PvPI) format | Case exercise: Type B ADR

A. Patient Information

Patient initials	R.K. (identity kept confidential)
Age	34 years
Gender	Male
Weight	68 kg
Hospital / Reg. No.	GH/OPD/2026/04521

B. Suspected Adverse Reaction

Date of onset	03/10/2026
Date of recovery	07/10/2026
Reaction / Event	Generalized pruritic rash (urticaria) with swelling of lips and eyelids (angioedema)

Description. A 34-year-old man presented to the outpatient department with sore throat and fever of two days' duration and was diagnosed with acute bacterial pharyngitis. He was prescribed amoxicillin-clavulanic acid 625 mg three times daily for five days. About three hours after the first dose he developed intense itching over the trunk and limbs, followed by raised erythematous wheals. By the next morning his lips and eyelids were swollen. There was no breathlessness, wheeze, hypotension or loss of consciousness. He had no known drug allergy and no previous exposure to this drug. The reaction occurred at a normal therapeutic dose and was independent of the amount taken, which is characteristic of a Type B (hypersensitivity) reaction.

C. Suspected Medication

Drug	Dose / Route	Frequency	Started	Stopped	Indication
Amoxicillin + clavulanic acid (Augmentin 625)	625 mg, oral	TID	03/10/2026	03/10/2026 (after 1st dose)	Acute bacterial pharyngitis

Manufacturer / Batch / Expiry	GlaxoSmithKline Pharmaceuticals Ltd. / Batch No. AC-26-0417 / Expiry 08/2027
Action taken	Drug withdrawn
Dechallenge	Positive: reaction subsided after withdrawal and treatment
Rechallenge	Not done (ethically avoided)

D. Concomitant Medications

Paracetamol 500 mg SOS for fever, started 02/10/2026 and taken for one day before the reaction. Not suspected, as the patient had tolerated it previously.

E. Relevant History and Investigations

Past history	No allergy, asthma or atopy; no previous adverse drug reaction
Family history	Non-contributory
Vitals at presentation	BP 118/76 mmHg, pulse 88/min, SpO_2 98%, afebrile
Investigations	Total eosinophil count mildly raised; liver and renal function tests normal

F. Management and Outcome

Management	Amoxicillin-clavulanate stopped immediately. Tab. cetirizine 10 mg once daily for 5 days; Tab. prednisolone 20 mg once daily for 3 days; calamine lotion for local application. A macrolide was substituted for the pharyngitis.
Advice	Avoid all penicillin-group drugs in future
Outcome	Recovered completely on 07/10/2026

G. Seriousness of the Reaction

Death	Life-threat.	Hospital.	Disability	Congenital	Non-serious
[ ]	[ ]	[ ]	[ ]	[ ]	**[X]**

H. Causality, Severity and Preventability Assessment

WHO-UMC causality: PROBABLE / LIKELY. The event followed a plausible time relationship with drug intake, is unlikely to be explained by the disease or other drugs, and improved on withdrawal. Rechallenge was not performed.

Naranjo scale question	Score
Previous conclusive reports on this reaction	+1
Event occurred after the drug was given	+2
Improved on stopping the drug	+1
Reappeared on readministration	0 (not done)
Alternative causes present (none found)	+2
Placebo response, drug level, dose relationship, similar past reaction, objective confirmation	0
Total	**6 = Probable (5-8)**

Severity (Hartwig and Siegel)	Level 3 (moderate): drug withdrawn and specific treatment required; no hospital admission
Preventability (Schumock and Thornton)	Not preventable: no previous exposure or allergy history
ADR classification	Type B (bizarre): unpredictable, not dose dependent, immune mediated (Type I hypersensitivity)

Report to the nearest ADR Monitoring Centre under PvPI (IPC, Ghaziabad). Helpline: 1800-180-3024. Patient identity is kept strictly confidential.